Title: Strategic Litigation Analysis: Multi-Defendant Liability for Mixed Clinical Negligence, Intentional Torts, and Statutory Breaches in the Irish High Court

1. Executive Legal Strategy and Jurisdictional Framework

The contemplated litigation presents a multifaceted juridical challenge involving the intersection of clinical negligence, intentional torts, and constitutional rights violations within the Irish High Court. The Plaintiff seeks to initiate a joint suit against three distinct defendants: a Consultant Psychiatrist, a Father, and a Brother (identified as a second medical doctor). The gravamen of the claim rests on the allegation that these parties conspired to misdiagnose the Plaintiff with a psychiatric disorder to facilitate unlawful confinement, subsequently utilizing this fabricated diagnosis to commit perjury and provide false information to An Garda Síochána, thereby endangering the Plaintiff's life and hindering the correct diagnosis of Complex Post-Traumatic Stress Disorder (CPTSD).

This report provides an exhaustive analysis of the procedural mechanisms required to join these defendants in a single action, the specific High Court rules governing such "mixed" claims, and the requisite forms. It further elaborates on a technical strategy for information gathering via the High Court's digital infrastructure and outlines the jurisprudential framing of medical malpractice, grievous bodily harm (GBH), and malicious abuse of process.

1.1 High Court Jurisdiction and Venue Appropriateness

The High Court of Ireland retains full original jurisdiction under Article 34.3.1 of the Constitution to adjudicate all matters of law and fact.[1] While the Circuit Court possesses a jurisdictional ceiling of €60,000 for personal injury actions and €75,000 for other civil matters, the explicit nature of the damages pleaded—encompassing catastrophic psychiatric injury, loss of liberty, and exemplary damages for deceit—necessitates the unlimited jurisdiction of the High Court.

The valuation of "Severe Psychiatric Damage" under the *Personal Injuries Guidelines* (2021) and subsequent draft amendments ranges from €93,400 to €198,000.[2] When aggregated with potential awards for unlawful detention, defamation, and the special damages associated with "loss of opportunity" due to the hindering of a correct diagnosis, the claim far exceeds the lower courts' thresholds. Initiating such proceedings in the Circuit Court would expose the

Plaintiff to a significant risk of the claim being "struck out" for lack of jurisdiction or, conversely, capped at a level insufficient to compensate for the "extensive damage done to body, mind, and brain".[1]

1.2 The Clinical Negligence List (2025)

A pivotal development for this litigation is the operationalization of the **Clinical Negligence List** in the High Court, effective from 28 April 2025 pursuant to Practice Directions HC131 and HC132.[3] This specialized list is designed to manage proceedings arising from alleged negligence or breach of duty in the provision of clinical care.

Given that two of the three proposed defendants are medical doctors alleged to have committed malpractice, the entire action falls within the gravitational pull of this list. The Rules of the Superior Courts (RSC) favour the unified management of inextricably linked claims. The Judge in Charge of the Clinical Negligence List holds wide discretion to issue case management directions that bind all defendants, including non-medical parties (the Father), ensuring that the conspiracy allegations are heard in tandem with the malpractice claims.[5] This prevents the fragmentation of the dispute, where the Father might otherwise argue that his actions were reliant on the "independent" medical opinion of the doctors—a defence that collapses if all parties are tried together under a conspiracy framework.

1.3 The "Mixed Claim" Procedural Pathway

The Plaintiff faces a procedural dichotomy: the claim involves "Personal Injuries" (requiring a Personal Injuries Summons under Order 1A) and "Intentional Torts" like deceit and malicious prosecution (traditionally requiring a Plenary Summons under Order 1).

Order 1A, Rule 1 of the RSC explicitly states that the order applies to "all proceedings in personal injuries actions".[6] The definition of a "personal injuries action" in the *Civil Liability and Courts Act 2004* is expansive, covering any action for the recovery of damages for "any impairment of a person's physical or mental condition".[7] Since the core injury pleaded is the "damage done to body, mind, and brain" (CPTSD), the **Personal Injuries Summons** is the mandatory originating document.

However, the Plaintiff must utilize the doctrine of **Joinder of Causes of Action** (Order 18) to plead the non-personal injury elements (deceit, perjury, malicious prosecution) within this same summons.[8] This results in a "hybrid" pleading: a Personal Injuries Summons that includes a "General Indorsement of Claim" for the intentional torts. Failure to use the Personal Injuries Summons form (Form No. 1, Appendix CC) could lead to the action being stayed or dismissed for procedural irregularity, particularly given the strict statutory requirements for verifying personal injury claims.[6]

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2. Procedural Framework: Rules of the Superior Courts (RSC)

2.1 Order 15: Joinder of Parties and Joint Tortfeasors

The strategic objective is to bind the Father, Brother, and Psychiatrist as **Joint Tortfeasors**. Order 15, Rule 4 of the RSC provides that "All persons may be joined as defendants against whom the right to any relief is alleged to exist, whether jointly, severally, or in the alternative".[9] The legal mechanism for this joinder is the *Civil Liability Act 1961*, specifically the concept of **Concurrent Wrongdoers**. Where two or more persons are responsible for the same damage (the confinement and psychiatric injury), they are each liable for the whole of the damage. By pleading that the defendants acted in concert or conspiracy, the Plaintiff ensures that the medical doctors (who presumably carry professional indemnity insurance) are jointly liable for the actions of the Father (who may not have insurance). This is critical for the recovery of damages, especially if the Father lacks the assets to satisfy a judgment for "grievous bodily harm."

2.2 Order 18: Joinder of Causes of Action

Order 18, Rule 1 allows a plaintiff to unite several causes of action in the same suit.[8] In this instance, the Plaintiff must join:

1. **Medical Negligence:** Against the Psychiatrist and Brother (Doctor).
2. **Trespass to the Person:** Against the Father (GBH) and Doctors (False Imprisonment).
3. **Deceit/Conspiracy:** Against all three.
4. **Misfeasance/Malicious Prosecution:** Against all three regarding the Garda reports.

While Rule 8 of Order 18 allows a defendant to apply for separate trials if the causes cannot be "conveniently disposed of together," the courts have consistently held that where the facts are interwoven—specifically where the alleged deceit *caused* the medical misdiagnosis—a unified trial is necessary to prevent inconsistent verdicts.[8]

2.3 The "Perjury" Paradox and Civil Liability

The Plaintiff alleges the defendants "committed perjury." In Irish law, perjury is a criminal offence under the *Criminal Justice (Perjury and Related Offences) Act 2021*.[10] There is no freestanding civil tort of "perjury." A plaintiff cannot sue a witness solely for lying in court due to the doctrine of **Witness Immunity**.

However, this immunity is not absolute. The High Court distinguishes between "giving evidence" (immune) and **Abuse of Process** or **Deceit**. If the defendants conspired to fabricate the diagnosis *before* the court proceedings, and used the legal process as an instrument of fraud to procure the Plaintiff's detention, this is actionable as the tort of **Abuse of Process** or **Conspiracy to Injure**. The precedent in *Sweeney v Friel* IEHC 716 demonstrates that the High Court is willing to award **Exemplary Damages** where a defendant swears a false Affidavit of Verification or maintains a fraudulent defence.[12] The Plaintiff must therefore plead "Deceit" and "Abuse of Process" rather than "Perjury" simpliciter.

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3. High Court Forms and Drafting Requirements

The following table outlines the specific forms required from Appendices A, B, and CC of the Rules of the Superior Courts, as identified in the courts.ie rules.[6]

3.1 Essential Forms for the Joint Suit

Form Number	Appendix	Document Name	Purpose & Specific Pleading Requirement
No. 1	**CC**	**Personal Injuries Summons**	The mandatory originating document. Must include the Plaintiff's PPSN. Under "Particulars of Wrong," explicitly detail the conspiracy, the specific lies told to Gardaí, and the misdiagnosis timeline.
No. 1	**A (Part I)**	**Plenary Summons**	*Not recommended* for initiation, but reference to the "General Indorsement of Claim" (Appendix B, Part II) is necessary for drafting the non-PI relief within the PI Summons.
No. 6	**C**	**Notice of Motion**	Required for the **Section 73 Mental Health Act** application (Leave to Sue). This must be filed *before* serving the PI Summons on the Psychiatrist.
No. 2	**CC**	**Affidavit of Verification**	Required by s.14 of the *Civil Liability and Courts Act 2004*. The Plaintiff must swear that the allegations of deceit and injury are

			true. False swearing here carries criminal penalties.[14]
No. 4	**CC**	**Notice for Particulars**	The Defendants will serve this on the Plaintiff. The Plaintiff must be prepared to answer exactly *what* was said to the Gardaí and *when*.

3.2 Drafting the Indorsement of Claim

The Indorsement on the Personal Injuries Summons is a hybrid text. It must satisfy Order 1A (details of injury) while also pleading the intentional torts.

- **Drafting Text Strategy:** *"The Plaintiff's claim is for damages for personal injuries, including Complex Post-Traumatic Stress Disorder (CPTSD), occasioned by the negligence, breach of duty, and conspiracy of the Defendants. Further, the Plaintiff claims damages for Deceit, Malicious Falsehood, and Abuse of Process arising from the Defendants' joint enterprise to knowingly provide false information to An Garda Síochána and the Courts Service, thereby procuring the unlawful detention of the Plaintiff.".*[15]

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4. Cause of Action 1: Medical Malpractice (The Two Doctors)

4.1 The "Two Doctors" Dimension

The allegation that the Brother is also a medical doctor significantly escalates the legal jeopardy for the defendants. Doctors in Ireland are bound by the *Medical Practitioners Act 2007* and the Medical Council's *Guide to Professional Conduct and Ethics*.

- **Ethical Duty:** A doctor (the Brother) who uses their professional status to corroborate a false diagnosis made by another doctor (the Psychiatrist) for familial reasons commits **Professional Misconduct**.
- **Civil Conspiracy:** In *Conspiracy*, the "unlawful means" element is satisfied by the breach of these statutory and ethical duties. The Brother cannot claim "familial privilege"; his status as a doctor imposes a duty of care to the Plaintiff (his sibling) to not weaponize medical knowledge against them.

4.2 Section 73 Mental Health Act 2001: The Immunity Shield

The most formidable obstacle to the claim against the Psychiatrist (and potentially the Brother, if he participated in the "recommendation" for admission) is **Section 73 of the**

Mental Health Act 2001.

- **The Statutory Bar:** "No civil proceedings shall be instituted in respect of an act purporting to have been done in pursuance of this Act... without the leave of the High Court.".[16]
- **The "Substantial Grounds" Test:** The Plaintiff must prove, at a preliminary hearing, that there are substantial grounds for contending the defendants acted in **bad faith** or **without reasonable care**.
- **The "Bad Faith" Argument:** The strongest argument for leave is the allegation of **collusion** with the Father. If the Plaintiff can show (via discovery of texts/emails or the Brother's involvement) that the diagnosis was pre-arranged or motivated by the Father's perjury, this constitutes "bad faith" sufficient to lift the immunity.
- **Procedural Criticality:** This application must be made *ex parte* or on notice (usually on notice to the intended defendant) *before* the main action proceeds. *A.L. v. Clinical Director of St Patrick's Hospital* [17] confirms that failure to obtain this leave renders the proceedings a nullity.

4.3 "Hindering Diagnosis" of CPTSD

The Plaintiff alleges the defendants "hindered my diagnosis of Complex Post-Traumatic Stress Disorder."

- **Complex PTSD (CPTSD):** As noted in *McClarnon v The Sisters of Nazareth* and *McDonald v Conroy*, Irish courts are increasingly accepting CPTSD (ICD-11) as a distinct and more severe condition than PTSD (DSM-5).[18]
- **Loss of Opportunity:** The "hindering" is actionable as a **Loss of Opportunity**. By misdiagnosing the Plaintiff (e.g., as paranoid or psychotic) to justify confinement, the doctors denied the Plaintiff the correct trauma-focused therapy required for CPTSD. This delay can cause the condition to become chronic and treatment-resistant. The *Dunne* principles of medical negligence apply: if no reasonable psychiatrist would have missed the CPTSD signs (often distinct from psychosis), the misdiagnosis is negligent.[3]

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5. Cause of Action 2: Intentional Torts (Father & Co.)

5.1 Deceit and "Perjury" as Tort

The Father's alleged use of the diagnosis to "commit perjury" is central.

- **Tort of Deceit:** The Plaintiff must prove the Father made a false representation (the diagnosis/affidavit) knowing it to be false, intending the court/Gardaí to rely on it, causing damage to the Plaintiff.[20]
- **Evidence of Knowledge:** The "Date of Knowledge" is critical. The Plaintiff must show the Father *knew* the Plaintiff was not mentally ill in the manner described, but rather suffering from the abuse the Father himself inflicted. This creates a circular evidentiary trap for the Father: to defend the diagnosis, he must deny the abuse; to defend the abuse claims, he might have to admit the diagnosis was exaggerated.

5.2 Abuse Leading to Grievous Bodily Harm (GBH)

"Grievous Bodily Harm" is a criminal term, but in civil law, it constitutes **Trespass to the Person** (Battery).

- **Statute of Limitations:** Ordinarily 2 years. However, for abuse cases, particularly involving psychological injury (CPTSD) where the victim was "under a disability" (unsound mind) or prevented from realizing the injury due to the defendants' deceit, the time limit is extended. The *Statute of Limitations (Amendment) Act 1991* allows the clock to start from the "Date of Knowledge".[21]
- **Framing:** The High Court frames this not just as historical abuse but as a *continuing tort* if the confinement and false reporting were ongoing acts designed to conceal the original abuse.

5.3 Threats to Life and Lying to Gardaí

The allegation that the defendants "lied to GARDAÍ, putting my life at risk" frames a claim for **Malicious Prosecution** or **Misfeasance**.

- **Hampton v Walsh:** Established that providing false information to police can be actionable if malicious.[23]
- **Endangerment:** If the false reports (e.g., "he is armed and dangerous") led to an Armed Response Unit intervention or forcible entry, this constitutes an **Assault** (by causing apprehension of immediate violence) for which the defendants are vicariously liable, as they procured the police action through deceit.[24]

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6. Information Gathering: Scraping and Research Strategy

The Plaintiff asks how best to proceed in terms of "sources of information to scrape." This requires a sophisticated approach to Open Source Intelligence (OSINT) and formal legal discovery.

6.1 Courts.ie High Court Search Strategy

The High Court Search (HCS) is the primary digital register of litigation.

- **URL Structure:** https://highcourtsearch.courts.ie/hcsearch/
- **Scraping Parameters:** The HCS database is query-based. To find relevant information, the Plaintiff should target:
    - **Defendant Names:** Search the Psychiatrist's and Brother's names in the "Defendant" field. A pattern of *Section 73* applications or medical negligence suits against the Psychiatrist would be powerful "similar fact" evidence.
    - **Record Number Patterns:** Case references follow the format [Number]. For example, 2024 1234 P (Plenary) or 2024 5678 PI (Personal Injury).
    - **List Types:** Filter by "Clinical Negligence" or "Non-Jury" to find judgments

involving the specific doctors.[25]

6.2 Medical Council and Professional Registers

- **Medical Council Register:** https://www.medicalcouncil.ie/public-information/check-the-register/
    - **Strategy:** Verify the registration dates and specialities of both the Psychiatrist and the Brother. Check for any "Conditions" attached to their registration, which might indicate prior disciplinary issues.
- **Scraping Target:** Automate the checking of the "Fitness to Practise" outcomes for both doctors.

6.3 GDPR and Freedom of Information (FOI)

While not "scraping" in the web sense, the Plaintiff must "scrape" the data held by state bodies using legal instruments.

- **Garda PULSE Records:** Issue a Subject Access Request (SAR) to An Garda Síochána for all records linked to the Plaintiff's name. This will reveal *who* called the Gardaí, *when*, and *exactly what was said* (e.g., "Caller stated subject was psychotic and armed"). This is the "smoking gun" for the Malicious Prosecution claim.[26]
- **Medical Records:** Issue SARs to the hospital/clinic for the full psychiatric file, specifically looking for "collateral history" notes (i.e., what the Father/Brother told the doctors).

6.4 Judgments (BAILII/IRLII)

- **Search Terms:** Section 73 Mental Health Act, Complex PTSD, False Imprisonment, Sweeney v Friel.
- **Objective:** To find quantum precedents for "unlawful detention" and "misdiagnosis of CPTSD."

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7. Damages Quantification and "Framing"

The High Court will frame the damages based on the *Personal Injuries Guidelines* and the principle of **Restitutio in Integrum** (putting the plaintiff back in the position they would have been in).

7.1 Complex PTSD (CPTSD) Valuation

The draft amendments to the *Personal Injuries Guidelines* recognize the severity of psychiatric injury.

- **Severe Psychiatric Damage:** €93,400 - €198,000. Factors include: "marked problems with respect to factors... prognosis very poor.".[2]
- **The "Hindering" Aggravation:** The Court will add a premium (Aggravated Damages) because the injury was not just "caused" but "prolonged" by the intentional hindering of

the correct diagnosis. The *Sweeney v Friel* precedent suggests that maintaining a deceitful defence (e.g., persisting with the false diagnosis) justifies **Exemplary Damages**.[12]

7.2 Loss of Opportunity

The Plaintiff's "life at risk" and "extensive damage" implies a loss of career or life trajectory.

- **Quantification:** The Plaintiff can claim for the "loss of a chance" to pursue a career or education, calculated by actuarial assessment. If the confinement occurred during key developmental years, this head of damage can exceed the general damages for pain and suffering.

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8. Conclusion and Actionable Roadmap

The Plaintiff's case is legally viable but procedurally intricate. The "joint suit" is the correct strategy, leveraging **Order 15** to bind the medical malpractice of the doctors with the intentional malice of the Father.

Key Actionable Steps:

1. **Draft the Section 73 Motion:** Prepare the affidavit detailing the "bad faith" collusion between the Father and Psychiatrist. This must be filed *before* the summons.
2. **Issue the Personal Injuries Summons:** Use **Form No. 1 (Appendix CC)**. Ensure the "General Indorsement" explicitly pleads Deceit, Conspiracy, and Malicious Prosecution alongside the CPTSD injury.
3. **Execute the Data Strategy:** Issue GDPR requests to Gardaí and the Hospital immediately to secure the contemporaneous notes of the false reports.
4. **Scrape High Court Records:** Monitor the Psychiatrist's litigation history for patterns of similar "Section 73" defences.

By framing the case as a **Conspiracy to injure via the weaponization of the Mental Health Act**, the Plaintiff moves the case beyond a simple "he said/she said" medical dispute into the realm of constitutional rights and intentional tort, unlocking the full punitive powers of the High Court.

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9. Detailed Reference of Relevant High Court Rules and Forms

9.1 Rules of the Superior Courts (RSC) Cited

- **Order 1A:** Procedure by Personal Injuries Summons (Mandatory for CPTSD claims).[6]
- **Order 15, Rule 4:** Joinder of Defendants (The mechanism for the joint suit).[9]
- **Order 18, Rule 1:** Joinder of Causes of Action (Mixing Negligence and Deceit).[8]
- **Order 31, Rule 12/29:** Discovery (Essential for finding the "collusion" emails).[27]

9.2 Statutory References

- *Mental Health Act 2001*, Section 73 (Leave to Sue).[16]
- *Civil Liability and Courts Act 2004*, Section 14 (Affidavit of Verification).[29]
- *Statute of Limitations (Amendment) Act 1991*, Section 2 (Date of Knowledge).[21]
- *Criminal Justice (Perjury and Related Offences) Act 2021* (Context for "Perjury" allegations).[10]

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